

Provider Initiated HIV Testing and Counseling

Participant Manual

ACKNOWLEDGEMENTS

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ABOUT CHART

The Caribbean HIV/ AIDS Regional Training (CHART) Network was established in 2003 for the purpose of contributing to systematic capacity development among institutional and community-based health care workers involved in prevention of HIV/ AIDS and in care, treatment and support of persons living with HIV and AIDS. The CHART Regional Coordinating Unit (RCU) is located at the University of the West Indies (UWI) and reports to the Vice-Chancellor. The RCU serves six national training centres, one each in the Bahamas, Barbados, Jamaica and Trinidad & Tobago, as well as two in Haiti. The OECS HIV/ AIDS Project Unit also belongs to the Network.

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Training/ General Objectives

Discuss how the provider's attitudes, values and prejudices may impact his/her counseling sessions with clients

Understand the key tasks to be accomplished when using the PITC protocol as a guide during counseling

Provide brief, targeted health education on HIV prevention and transmission, as well as HIV treatment and care;

Appreciate the need for confidentiality and consent and how to employ these in counseling environment.

Deliver HIV test results to patient and reporting to the National HIV /STI Programme (NHP).

Background and Rationale for PITC

Since HIV antibody testing first became available, World Health Organisation (WHO) has advocated for persons at risk for HIV to voluntarily seek out HIV testing and counseling. The cornerstone of WHO's guidance on HIV testing has remained constant for nearly twenty years: confidentiality, informed consent, and access to quality counseling. Throughout the World, programs offering voluntary counseling and testing (VCT) have been successful in many countries in providing individuals with knowledge about HIV, prevention measures, and in providing HIV test results to millions of individuals. Still, in Jamaica up to 50% of individuals estimated to be infected, are not aware that they are infected with the HIV. Reaching individuals living with HIV who do not know their serostatus must clearly be a global public health priority.

In light of this, coupled with advances in treatment and care, and following the WHO and UNAIDS the Jamaican NHP has advocated for an increase in Annual Routine opt-out provider-initiated HIV testing and counseling (PITC) at all levels of health delivery in public and private settings. The recommendation for *universal systematic* offer of HIV testing and counseling is seen as an important step to achieving the goal of universal access for all persons with HIV/AIDS. This intervention provides an entry point into HIV treatment, care and support behavior change counselling and provides a seminal opportunity for HIV prevention.

It should be noted that the position of WHO/UNAIDS on provider-initiated HIV testing and counseling describes two discreet categories:

- **Diagnostic Testing:** refers to situations where a medical condition [e.g., TB] or medical symptoms [e.g., opportunistic infections or unexplained significant weight loss] indicate a significant possibility of underlying HIV disease; and
- **Routine Recommendation:** refers to recommending in settings where potential benefit is especially high [antenatal settings] or where a high probability of HIV infection exists [e.g., high prevalence regions, STI treatment settings].

An important caveat in the shift to provider-initiated testing is the importance of having mechanisms in place to assure referral to post-test counseling services to all patients, and referrals to medical and psychosocial support for those testing positive. In addition, moving to an 'opt-out' model of obtaining consent (e.g., "Unless you object, I plan to administer an HIV test) removes many of the sociocultural barriers that often impede access to HIV testing service and engagement with the Healthcare system. **Every effort must be made to ensure voluntary informed consent to HIV testing.** WHO/UNAIDS have consistently refuted any benefit to mandatory testing, and ensuring consent is a principle that cannot be overlooked.

Finally, health workers should remain aware of the power--and potential status--of the health care provider in their setting and in their community. The recommendation for HIV testing will be, for some patients, tantamount to an edict. Sensitivity to patient volition and recognition of the right to refuse testing should guide a provider's interactions in obtaining consent.

When moving to routine offer, the standard pre-test counseling components used in VCT are adapted to simply ensure informed consent, without a full education and counseling session. However, it is an expectation that every effort will be made to identify additional support within the health care setting for education and emotional support as part of the HIV post-testing interaction for those who may exhibit/request additional support requirements.

Finally, this course does not attempt to meet the needs of all clinicians doing routine recommendation of testing for diagnostic purposes. WHO and the U.S. CDC have jointly produced curricula that deal with testing in TB and ante-natal settings. The primary utility of the materials in this training package are for clinicians treating STIs, providing family planning services, and in general medical settings with high HIV prevalence. It is also important to acknowledge that this course does not deal with some important clinical information which would need to be part of a course on diagnostic testing for suspicion of HIV infection. The training that most closely meets that need would be the Integrated Management of Adult/Adolescent Illness (IMAI) courses developed and delivered by WHO.

Primary Goal of PITC Course

A primary goal for this course is to increase the capacity of health care providers to offer HIV testing to patients seeking care in public hospitals and health facilities.

In the development of these materials, an important focus has been on adapting the model of HIV testing and counseling to:

- address health care providers' concerns about time and staff constraints in busy health care delivery settings;
- support the implementation of an approach to ensure that a greater number of persons know their status and access treatment; and
- offer clear and direct suggestions on achieving goals for provider initiated testing and counseling.

Course Description

This training has been designed to offer a model (see overleaf) of HIV testing in which the provider recommends HIV testing with 'opt-out' language, assures confidentiality, and obtains informed consent. The model offers a graphic comparison between a typical VCT intervention and a typical PITC intervention.

The pre-test information and education given may be offered by any member of the health care team. In this model, the education session facilitates the patient making an informed decision. The provider then confirms the patient's desire to test. Finally, the test is performed and results are delivered with posttest counseling focused on providing access to treatment and partner counseling services if positive, and with any necessary referrals. The provider-initiated opt out approach yields a high uptake in testing. Critical to this process however is providing the patient with an opportunity to opt out if they so desire, though providers should assess patient's barriers to testing and create a plan with the interdisciplinary team to overcome this resistance.

In Jamaica, the NHP has adopted the use of Rapid HIV testing, utilizing validated algorithms that allow for same-day testing and delivery of confirmed HIV status to the patients. The PITC opt-out approach is a key intervention to improving coverage rates of HIV screening, increased engagement in HIV-related services and reducing healthcare burdens associated with late-stage HIV diagnosis.

I. Provider -Initiated Testing and Counseling (PITC) Overview

Learning Objectives:

Knowledge

- Identify the similarities and differences between PITC and VCT

Main topics:

Overview of PITC

Comparing PITC and VCT

Provider -Initiated HIV Testing

Throughout the Caribbean there is an urgent need to ensure that people living with HIV have access to treatment, care, support and prevention. This is not possible if the persons infected – and the health care system that serves them – is not aware of their HIV status and their needs. Health Care systems need to adopt an efficient and effective system for increasing the number of persons who have the opportunity to do an HIV test. There is a need therefore to consider a shift to the Provider Initiated Approach to achieve a high uptake in HIV testing and counseling. This method has been used in public health settings to achieve this objective of high yield in cases of other health conditions; such as reducing syphilis in pregnant women, increasing immunization, prevention of various cancers.

Benefits of PITC

Time: P-I testing requires less of primary care providers' time as counseling expectations in VCT are waived. The differences between VCT and PITC will be looked at later in the course

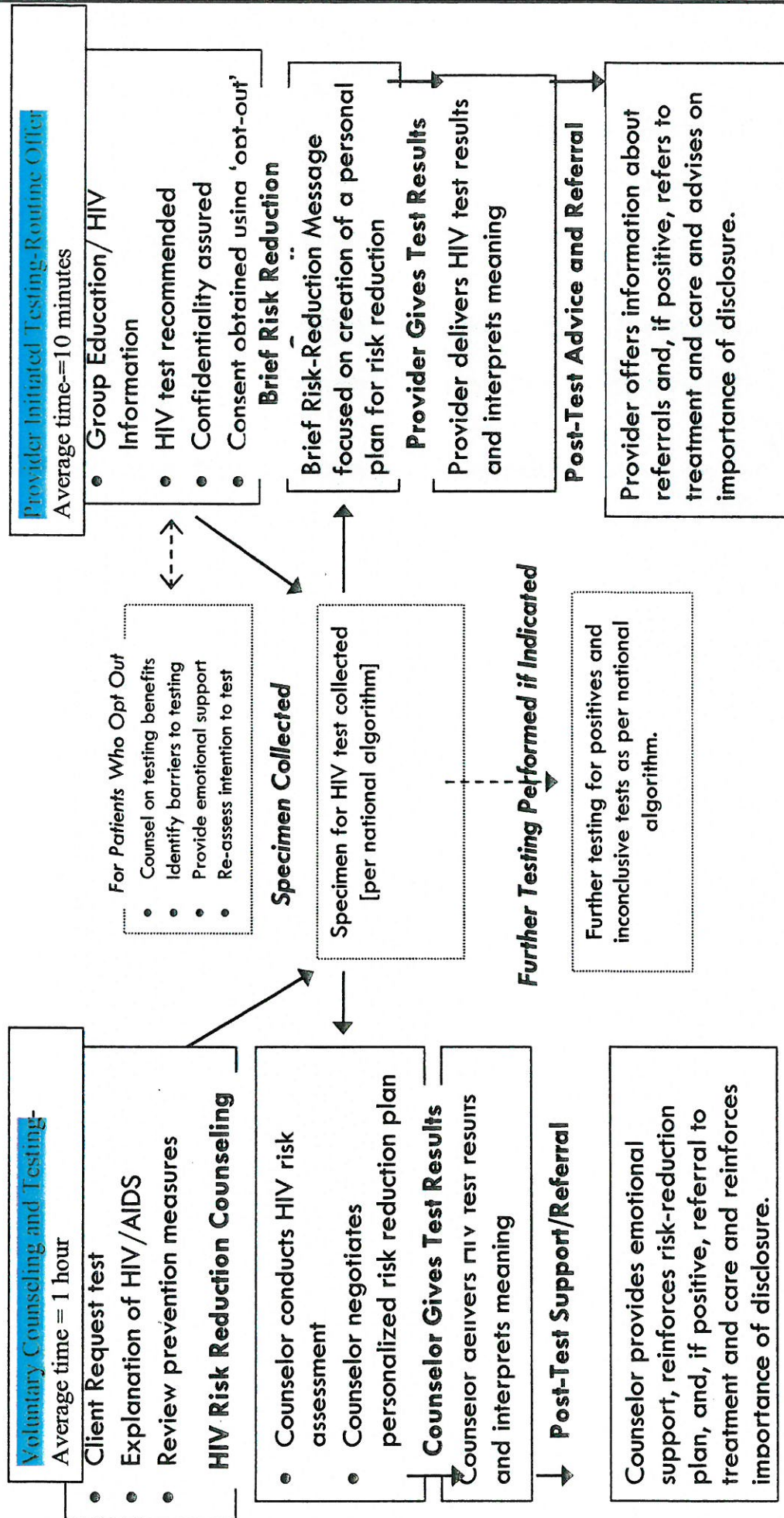
Human Resources: Uses other staff to assist with HIV prevention tasks. These may include existing staff and trained lay counselors.

Stigma: Providers may not be excited about offering HIV testing due to the discomfort they anticipate patients may encounter. If patients feel 'singled out' or somehow stigmatized because HIV testing is offered, it may hurt the patient-provider relationship. However, if this service is routinely offered to all or most patients present in the particular environment then, the procedure and offering of the procedure becomes normalized.

Multiple Patient Needs: Some providers may be resistant to offering HIV testing because of a perception that they were to 'fix' all of the emotional and other problems facing patient. With a de-emphasis on counseling and emotional support, the testing intervention becomes a more typical medical screening procedure and linking patients to appropriate counseling and support becomes a function of effective referral not intensive individual counseling in the health care setting.

Differences and similarities between Voluntary Testing and Counseling and Testing (VCT) and Provider Initiated Testing and Counseling (PITC) Source: *Caribbean Guidelines for the Scale Up of HIV Testing and Counselling Towards Universal Access*

COMPARING PROVIDER-INITIATED AND VOLUNTARY HIV TESTING AND COUNSELING



II. Benefits and Barriers to Testing

Learning Objectives:

Knowledge:

- Identify barriers that may impact on the level at which individuals utilize HIV testing services
- Identify the benefits to be gained from doing a HIV test
- Identify the three Cs that guide all models of HIV testing and counseling

Attitude

- Recognize how confidentiality and informed consent can impact an individual's health decisions

Main topics:

Benefits of HIV testing

Barriers to HIV testing

Confidentiality and consent

Benefits and barriers to HIV testing

What is HIV Testing?

HIV testing is the process by which blood or body fluids are tested for the presence of antibodies or the viral detection as markers that occur during the HIV infection.

There are four types of HIV testing:

- Voluntary counseling and testing
- Diagnostic HIV testing
- A routine offer of HIV testing by health care providers
- Mandatory HIV screening

The four types are clearly distinguished by UNAIDS/WHO. For more details make reference to WHO/UNAIDS policy on HIV testing found in the back of the manual.

Benefits of HIV Testing

- Knowledge of HIV status and prevention strategies to be employed.
- Early intervention into treatment, care and support services
- Sustained optimal health in a community by reducing morbidity and mortality rates of HIV disease
- Provide for programme planning, budgeting, monitoring and continuous education
- Reduce the burden and effects on the national economy.
- Reduce fear, stigma and discrimination
- Make informed choices concerning reproductive health

Activity 1: Benefits of HIV testing (Group 1)

Objective: Identify the benefits to be gained from doing a HIV test

Instruction: Imagine you are a patient thinking about taking a n HIV test, how would you benefit from doing the test?

1. _____

2. _____

3. _____

Barriers to HIV testing

- Fear of illness, death and abandonment.
- Attitudes of healthcare providers
- Stigma and discrimination among healthcare providers and others.
- Lack of knowledge of HIV disease and prevention methods.
- Denial of past/present risky behaviours
- Loss of job and family support.
- Community rejection
- Fear of violence
- Non-disclosures of partner's HIV status.

Activity 2: Barriers to HIV testing (Group 2)

Objective: Identify barriers that may impact on the level at which individuals utilize HIV testing services.

Instruction: Imagine you are a patient thinking about taking an HIV test, what would hinder you from not taking the HIV test?

1. _____

2. _____

3. _____

Stigma and Discrimination

Stigma - Negative thoughts or actions towards someone. Stigma brings with it shame, denial, isolation and distress.

Discrimination - Unfair treatment of someone because you think he or she is different in some way.

Unique features of offering HIV testing

- The negative consequences that is associated with it.
- Communities discovering someone with HIV is living in their community
- Prejudice, discrimination, and fear of the stigma of living with HIV are a huge barrier to utilizing HIV testing services.
- Healthcare workers should identify this as an important factor when offering HIV testing

Reducing Stigma and discrimination in the health and social sector

- Educate individuals about the disease
- Reach out to others. When you hear negative perceptions point them out.
- Don't equate an individual with their illness.
- Silence encourages feelings of shame. Include HIV/AIDS in discussions about acceptance of diversity.

Groups 1 & 2

Activity 3

Objective: Define stigma and discrimination

Instruction: Define in your own words the terms stigma and discrimination

Stigma

Discrimination

The WHO/UNAIDS HIV testing policy emphasizes the importance of including in HIV testing policies the '3 C's of HIV testing and counseling: **consent, confidentiality and access to counseling.**

Confidentiality and Consent

The WHO/UNAIDS HIV testing policy emphasizes the importance of including in HIV testing policies the '3 C's of HIV testing and counseling: **consent, confidentiality and access to counseling**

What is confidentiality?

Confidentiality is an important element of a care provider. This means that care providers should ensure, that all client information are kept private and their wishes as to whom personal information is share is respected.

Informed consent

Deliberate and autonomous permission granted by a client to perform a specific test (HIV test) or procedure

Due to the serious nature of HIV testing and counseling, it is of vital importance that a provider ensures that when a client or patient grants consent he or she in a rational state of mind.

Minimum information for consent

The least amount of information needed in order to provide informed consent is four (4)

- Clinical benefits and prevention benefits of testing and the risks
- Right to refuse
- Follow-up services that is available
- Encouragement of disclosure by a person tested as HIV-positive, result with past and future partner/s
- Opportunity to ask the health care provider questions
- Test result will be treated as confidential

Activity 4: Confidentiality and informed consent

Objective: Recognize how confidentiality and informed consent can impact an individual's health decisions

Instructions:

1. In pairs, discuss the question: "Where are potential situations or places in the clinic when a patient's confidentiality may be compromised" After discussion share your findings with the other member of the group.

2. Identify the different ways of determining consent while recommending HIV testing.

III. A Review of the Basic Skills for HIV Counseling

Learning Objectives:

Knowledge:

- Explain the basic skills for HIV counseling

Attitude

- Display how a provider's self-awareness, attitudes, values and prejudices may impact interaction with clients

Main topics:

Definition of counseling

Elements of counseling and testing

Skills for HIV counseling

Value, Attitudes and Prejudices: How they impact the HIV counseling interaction

Definition of counseling

Counseling is a two-way communication process between the sender (counselor) and the receiver (client). This process is aimed at helping individuals to explore personal issues, make their own decisions and action plans.

HIV and AIDS counseling is a confidential communication exercise between a client and a care provider. The exercise is aimed at assisting the client in making decisions relating to HIV and AIDS and how to prepare an effective and appropriate plan of action that will help the client to cope with stress.

Elements of counseling and testing involve:

Exercising good listening skills

Respecting clients differences

Showing concern about issues relating to HIV

Demonstrating good interpersonal skills

Showing compassionate

Providing a counseling environment that provide privacy for the client

(See Counseling and Testing for HIV Reference Manual for additional information)

Skills for HIV Counseling

Some of the skills that are used by providers when offering HIV testing and counseling services are:

- Empathizing
- Attending
- Open questioning
- Probing
- Focusing
- Affirming
- Challenging
- Correcting false information
- Summarizing

(See Counseling and Testing for HIV Reference Manual for additional information)

Value, Attitudes and Prejudices: How they impact the HIV counseling interactions

In general counselors have their own strengths, weaknesses, beliefs, attitudes, fears, values and worries. These may affect how they interact with clients they work with.

Self-Awareness

It is important to have an awareness of one may be impacted by others and how you affect others. Having this knowledge of self may aid in:

- Understanding personal feelings, thoughts, attitudes and beliefs, and how they can impact counseling, whether positively or negatively.

- Knowing that clients have their personal feelings, thoughts, attitudes and beliefs and they impact they may have on their ability to talk about any issues or concerns that they may have.
- Assessing their own behaviors and attitudes and be comfortable in asking the question: How am I doing? How can I improve?

Attitudes and Values

These are feelings, beliefs and emotions about a fact, thing, behavior or person. For example, some people believe that having many sexual partners is acceptable, while others believe it is unacceptable.

Counselors should be cognizant of the fact that attitudes and values may negatively influence their behavior toward clients who they themselves will portray different attitudes and values. It is vital that providers are cognizant of their own attitudes and values. If they are then they will be able to comfortably engage their clients without judging, criticizing or reprimanding them. Treating all clients with respect is the key.

Prejudice

A prejudice is a negative opinion or judgment made about a person or group of people without knowing the facts. Reacting without knowledge, leads without a doubt to prejudice.. Prejudices can prevent providers and clients from having open and honest discussions about sensitive issues. Prejudice can be manifested in many ways, some ways are:

- Prejudice against different racial and political groups
- Prejudice against vulnerable groups for the homosexuals and sex workers
- Prejudice against religious groups

V. Patient Education Strategies

Learning Objectives:

Knowledge:

Explain the different strategies for an effective HIV education session

Main topics:

Strategies for Effective HIV Education

Pre - test Education

Strategies for Effective HIV Education

Start education by asking what your patient already knows.

One simple strategy for assessing a patient's information gaps as well as verbal abilities can be to ask an open question like "What do you know about the risks for HIV in this village/your community?" A patient's response gives you valuable information about their existing knowledge, and allows providers to work more efficiently by not teaching patients what they already know.

Use simple, non-technical language; use terms your patient uses.

It is sometimes a difficult task for health care workers, to translate medical terms and concepts into simple language that the patient can understand. Health care provider to make every effort to language that is simple and free of technical terms and expressions.

You can sometimes provide health education on sensitive topics by discussing them in third-person language.

One strategy with shy patients or uncomfortable topics may be to say something like, "many of our patients find it really hard to disclose their status to their husbands/partners." The goal of health education is for patients to use information, and presenting information in a minimally threatening way is ideal.

Patients can retain an average of only three take-home messages in any health education intervention.

Ask if anyone has ever needed to be taught a new skill by someone who was trying to offer them too much information and too many choices. Confirm responses. Explain that health information is as foreign to our patients as that information may have been for many of us.

The skill for providers is to prioritize which health education issues are most important at this visit and to prioritize those while allowing the opportunity to return to other topics in subsequent visits.

Pre-test Education

In the case of PITC, the client is provided with pre-test information only; risk reduction counseling is not performed during the pre-test session. Depending on the local situations and policy pre-test information can be provided in the form of individual information sessions or in group information talks. However, informed consent must be obtained on an individual basis.

Elements of Pre-test Education

- Mechanics and logistic of testing and receiving results
- Facts about transmission and prevention
- Possible need for repeat testing
- Information about availability of treatment and care services if positive
- Importance of disclosure

It is very important that the client is exposed to pre-test information. Although there is a positive side to HIV testing and counseling, there may exist also, the negative consequences for the individual and the family. As such, the following should be reviewed with clients as part of pre-test information/counseling. These include:

- That those who test positive may be discriminated against
- Suffer abuse and violence from spouse/partners
- Be abandoned by family and friends
- May have serious emotional reaction to a positive result
- May lose their job

VI. Components of the PITC protocol

Learning Objectives:

Knowledge

- Explain the purpose of the PITC protocol
- Describe the different components of the PITC protocol
- Explain the key tasks to be achieved in the different components of the PITC protocol

Skill

- Demonstrate the ability to provide PITC counseling using the protocol as a guide

Main topics:

Overview of the PITC Protocol

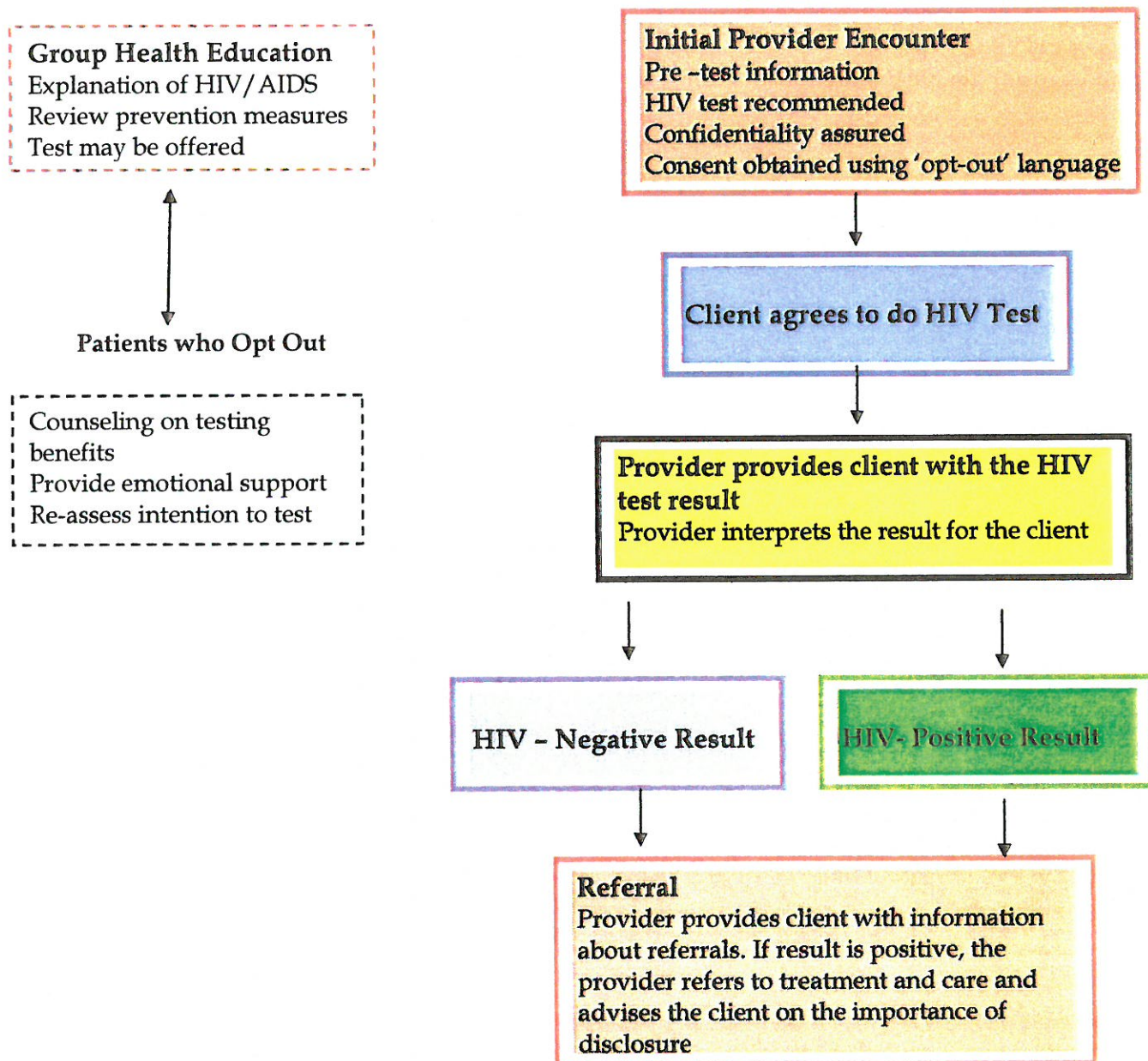
A model for PITC HIV Test

Components of the PITC protocol

Overview of the PITC Protocol

What is a protocol?

A protocol is a plan or process that assists individuals to correctly execute a task. In the same way the PITC protocol helps the health care provider by outlining the main tasks that they need to focus on and complete during their interaction with clients. The protocol also provides examples of questions that the provider can ask the clients so that the tasks can be accomplished.



Model for PITC HIV Test

Group Education/HIV Information

Group Education is optional and is dependent on the availability of staff and time. A brief explanation about HIV transmission, HIV testing procedure, benefits of testing and prevention measures may be done. This is a simpler and shorter intervention than normal individual pre-test counseling performed in VCT. Also, in some settings, this pre-test information may be given by another worker in the clinic in lieu of group education sessions.

Provider engages patient individually and recommends HIV test, assures confidentiality, and obtains consent.

The provider focuses on voluntary consent and assures confidentiality. Note that, where group education is not delivered, the educational components about HIV transmission and testing should be covered in this step

Specimen for HIV testing is obtained

HIV testing may be done using the rapid tests or Elisa tests. The type of test used will determine whether specimens will be obtained through finger stick or whether a blood draw will be required. The national protocol will determine whether results are given to clients/patients on the same day or whether they will be asked to return for their results. In either event, all facilities are expected to follow the country's algorithm for HIV testing.

If initial test is positive, a second sample should be obtained and new test performed

Provider gives the client the test result and referrals

The client is given their HIV test results, those results are explained, and referrals for ongoing support or other needs are made. While the results may be given by other clinic staff, primary care providers are encouraged to give HIV test results when this is feasible. Note here that the test was recommended by a health care provider with a distinct medical rationale for it. It is logical that these results be interpreted--and follow up plan of care as indicated--by the primary care provider who began the process.

Components of the PITC Protocol**Pre-test Information**

- Share the reasons why HIV testing is being recommended
- Explain the clinical and preventive benefits and risks of HIV testing
- Provide information on the services that are available whether the test is negative or positive
- Explain that there exists the right to decline HIV testing and that the test will be performed unless they object
- Share that declining HIV test will not impact access to services
- Explain the importance of disclosure to past and present partner(s) if the test result is positive
- Reassure the client that the test result is confidential
- Get verbal consent
- Invite client to ask you questions

Provide HIV- Negative Test Result**Post-test counseling with negative HIV result**

- Inform to the client that the test result is available and make the patient comfortable
- Ensure that you are dealing with the correct client, e.g. check code number and code name
- Assess readiness to receive result
- Review some of the issues covered in pre-test session, e.g. what HIV positive and negative mean, how the person anticipated coping with results
- Inform the patient of the result simply and clearly and give the patient time to consider
- Explain the test result, including information about the window period and the appearance of HIV antibodies and possible need for retesting
- Allow the person to talk about his/her feelings about the results, making sure that the person considers the implications of the result
- Ensure the person understands the meaning of the result
- Discuss the importance of staying negative and methods to prevent transmission of HIV
- Provision of condoms and guidance on their use
- Inform the person that counselling is available in the future, either alone or with a partner, close friend or relative

Provide HIV- Positive Test Result

Post-test counseling with positive HIV result

- Inform to the client that the test result is available and make the patient comfortable
- Ensure that you are dealing with the correct client, e.g. check code number and code name
- Assess readiness to receive result
- Review some of the issues covered in pre-test session, e.g. what HIV positive and negative mean, how the person anticipated coping with results
- Inform the patient of the result simply and clearly and give the patient time to consider. Allow the person to talk about his/her feelings about the result.
- Refer to the difference between HIV and AIDS
- Allow the patient to ask questions
- Help the patient to cope with emotions arising from the test result
- Discuss how the person is going to get through the next hours and days
- Discuss any immediate concerns and sources of support
- Describe follow-up services that are available in the health services and the community
- Provide information on how to prevent transmission of HIV, including the provision of male and female condoms and guidance on their use
- Provide information on other relevant health measures
- Discuss issues of disclosure of results, when and how this may happen and to whom
- Encourage and offer referral for testing and counselling of partners and children
- Assess risk of violence or suicide and discuss possible steps to ensure the physical safety patients, particularly women
- Arrange a specific date and time for follow-up visits or referrals for treatment, care, counselling, support and other appropriate services

Post-test counselling to pregnant women who are positive

- Talk about childbirth plans
- Use of antiretroviral drugs for the patient's own health, when indicated and available and to prevent mother-to-child transmission
- Discuss the need for adequate maternal nutrition, including iron and folic acid
- Inform the mother of infant feeding options
- Inform the mother of the HIV testing for the infant and follow-up that will be necessary

Referral

Referral letter- A referral letter establishes authority and assists the client to act on their own behalf. It should be noted that a referral is not a way to pass a problem along, but has a meaningful purpose for promoting the clients well-being.

Observe Confidentiality- In the course of making referrals, confidentiality must be observed. Patients should know the information about themselves that is included in making the referral. The referral letter which you are giving to the patient should not contain any information that you would not want them to know or can prove harmful to them if it is lost or misplaced..

Refer to known and trusted resources- The most effective referrals are to specific providers with whom you have had experience and who can deliver.

Offer referral as one option- The client is offered the referral as a possible resource for them to take advantage of.

Assess client's reaction to referral- Note that clients may have a history with agencies and providers, and some referrals may have negative stigma in client's minds. It is the client's right to refuse a referral, it is your responsibility to offer.

Instruct clients and prepare clients for agency services you are referring to- Suggest that specific teaching about what to bring, how to get to agencies, and what to expect at the organization will maximize the possibility that clients don't 'fall through the cracks' of the service plan.

Assess level of support for active referral- At times, client empowerment means giving client little direction and encouraging them to follow through, while other clients need to have appointments made or a plan of assistance developed.

Follow-up with client and referral source as appropriate: Find out from client and/or provider if client did follow through and how successful the interaction was.

Receiving referrals- How you receive and treat with a referred client will determine the success or failure of the intervention. It is important to acknowledge receipt.

Disclosure and partner notification: Your role is to assist clients who are HIV positive to understand the importance of partner notification and the importance of their present and past sex partners being tested if this has not already been done. It is imperative that providers assist clients to develop a plan for disclosure.

Using the PITC Protocol to conduct HIV Counseling

Learning Objective:

Skill:

Demonstrate the ability to provide counseling using the PITC Protocol

PRE-TEST INFORMATION

The health care worker should provide the patient the following information [at a minimum]:

- Reasons why HIV testing is being recommended

Say: "In order to understand your health problem, I need to know if it is related to you having HIV." ... *this should be used only in "diagnostic" circumstances ..where it is a one on one with the patientnot in the clinic....the person should not feel singled out... additionally I would restate it like this to make it less intimidating ...* ""

In order to understand your health problem, I need to know if it is related to your having HIV... it may not be..However... we need to check in order to provide you with the best care" ...

Today HIV..the AIDS virus, is spreading throughout our community ...and most people do not know when they have it ...but there is a lot we can do as medicines are available to keep people healthy.....so we are recommending that everyone does the HIV test...it is available today at the clinic...we will give you test todayplease let us know if you don't want to do it today...but otherwise we will give you the test for HIV today...

- Clinical and preventive benefits and risks of HIV testing

Say: "There are many things we can do if we find out you have HIV, including medicines which keep many patients healthy for a long time." or

"If you know you have HIV, you can protect yourself from other diseases, and keep your wife/husband safe... we at clinic can help you to understand HIV and if you wish help you to have sex safely ...and protect yourself from other infections."

- Services that are available whether the test is negative or positive, including availability of antiretroviral therapy

Say:

"We will offer drugs which fight HIV if you have the virus." or

"If you are negative...that is you don't have HIV ... we will treat your other health problems and we have counselors who can help you learn to stay negative and protect yourself."

- The right to decline HIV testing, and that the test will be performed unless they object.

Say: "I want to perform an HIV test today. If that isn't alright, you need to let me know." or

I think that we need to state this as a benefit to the patient ..e.g.

"I think this test will help me take care of your health and, unless you object, I'm going to obtain a sample. Can you agree with me?"

- Declining an HIV test will not limit the patient's access to services...*is this necessary*
Say: *"You will get medical care from us regardless of your decision about taking this HIV test."*
or
"If you refuse this test, we will still take care of you."
- The importance of disclosure to former partners if the test is positive
Say: *"It will be very important to let your former partners know that they may have been exposed to HIV if you are positive. We will help you with that if you like."* or
"If you test positive, notifying your former partners will be extremely helpful so they can take care of their health."

For women of reproductive age or who are pregnant, include:

- The risk of transmitting HIV to the infant
Say: *"A baby whose mother has HIV may be HIV + themselves."* Or
"A woman with HIV may give HIV to her baby."
- PMTCT interventions that are available
Say: *"The risk to the baby is greatly reduced if a woman finds out her HIV status early in her pregnancy."* or
"There are very good medicines which can protect a baby, but we must know a mother's HIV status to start these."
- Benefits to the infant of early diagnosis of HIV
Say: *"If we find out a baby has HIV early, we can take measures to keep the baby healthy longer."*
or
"There are important things which can help a baby whose mother has HIV, but we must know early in order to help."

Assure Confidentiality

- Reassurance the test result is confidential
Say: *"No one in the clinic will tell people you know about this test."* or
"We take strong precautions to make sure no one finds out about your results- or even if you have taken a test."

Informed Consent

- Patient's verbal consent and invite any questions
Say: *"Can you agree with my plan?"* or
"Is it alright for me to perform this test?"

If patient declines HIV testing, you may wish to identify barriers and strategize ways to overcome these barriers. Note also that some patient groups may be more susceptible to coercion to be tested or to adverse outcomes of disclosure of HIV status (e.g. violence, abandonment, incarceration). In such cases, providing additional information beyond the minimum requirements may be appropriate to ensure voluntary informed consent.

POST-TEST COUNSELLING

Provide HIV Negative Result

- An explanation of the test result, including information about the window period for appearance of antibodies and possible need for retesting
Say: *"This test is negative-HIV antibodies weren't found. Remember this test will not reflect any contacts you have had in the last three months. Can we talk about that?"* or
"If you have been exposed by a recent partner, we will need to schedule another test to be sure you didn't get HIV from that partner."
- Basic advice on methods to prevent HIV transmission
Say: *"Having one partner who you know is faithful and who is also negative is an excellent way to avoid HIV."* or
"If you aren't with only one partner or if you aren't sure about your partner's practices, using a latex condom every time you have sex is a great way to avoid HIV."
- Provision of male and female condoms and guidance for their usage
Say: *"Here are some latex condoms. Tell me what you have heard about how to use them."* or
"This is a female condom. You can decide when to use one-it will be your decision. Let me show you how it works."

Provide HIV Positive Result

- Inform patient of the result and give time to consider
Say: *"The test is positive-this means we did find HIV in your body."* or
"The results suggest you have been infected with HIV-you are HIV positive."
- Ensure patient understands
Say: *"What does what I have just said mean to you?"* or
"If you were going to explain to someone what I just told you, what would you say?"
- Allow patient to ask questions
Say: *"What would you like to ask me now?"* or
"Is there information I can offer that will be helpful to you?"
- Help patient cope with emotions
Say: *"This is really hard news to hear. Tell me about how you are feeling."* or
"How are you feeling now that you've heard this result?"
- Discuss immediate concerns and immediate sources of support
Say: *"What do you plan do to in the next 24 hours?"* or
"Let's talk about who could support you in this difficult time. Who do you think you can tell about this news?"
- Discuss follow up services available
Say: *"We have staff here providing medical treatment and support groups. I think both of these could help you."* or
"There is a doctor who provides specialized HIV care, and I want you to consult with him next time he will be in the village."
- Provide information on preventing transmission
Say: *"Remember how HIV is transmitted. It will be important now for you to not get your blood, semen, vaginal secretions in someone else's body."* or
"Abstaining from sex or using condoms every time you have sex are ways to protect yourself and other people."
- Provide information on relevant preventive health measures
Say: *"There are many things you can do to take care of yourself which may have a big effect on your future health. Can we talk about how healthy people stay healthy?"* or
"Eating right, exercising, taking medications are three important ways that people with HIV keep themselves healthy."
- Discuss issues of disclosure
Say: *"Letting your former partners know about your test will help them get tested themselves. This is a very important thing to do if we are going to stop HIV in our area."* or
"We have workers in our clinic who can confidentially notify your former partners. They will not name you, but just say that a former partner tested positive and they should be tested."

- Encourage referral for testing of children and partners
Say: *"This virus may have been passed to your children. I'd like us to make a plan to test them."*
or
"It is very important that your partner be tested. They may be negative and we can help him stay that way. Otherwise, I'd like to start him on medicines and take advantage of some of the things you are"
- Assess the risk of violence/suicide
Say: *"What are the reactions that you fear when you let people know your status?"* or
"What will your husband do when he hears you have tested positive for HIV?"
- Arrange appointments for follow-up services (e.g. counselling, family planning, STI treatment)
Say: *"I'd like to schedule an appointment to come back and see the nurse this week. She will do some tests, and decide what the best things we can do now to keep you well are."* or
"Since you may become pregnant and that could mean a risk that your baby would have HIV, I'd like to schedule an appointment for you to talk to our family planning nurse about birth control. For now, it might be a good idea for you to find a method and use one when you have sex."

Post-test Counseling to Pregnant Women who are Positive

- **Childbirth plans**
 Say: *"I'd like to talk about your options for delivering this baby. Being at home may not be such a good idea."*
"So that we can give you medicine and watch you carefully, we'd like to have you deliver the baby at hospital."
- **Use of antiretroviral drugs**
 Say: *"These drugs I'd like to prescribe will help you and your baby. It means a much lower risk for the baby getting HIV."* or
"If we start you on medicine, it will be like helping your baby stay well."
- **Infant feeding options**
 Say: *"Because there may be HIV in your breast milk, we want to recommend that you not feed the baby from your breast, but use a bottle to feed her. Have you known anyone who has done this?"* or
"There may be small amounts of HIV in your breast milk, but the water in the village can hurt the baby. I recommend you breast feed the baby until he is one year old, then we can talk about options for his nutrition."
- **HIV testing for the infant and necessary follow-up**
 Say: *"We will do testing for eighteen months on your baby to make sure he doesn't get HIV. It will be important that you keep appointments so we can test him."*
"You will come to the clinic every 3 months with the baby so we can keep her well. We will monitor her health and treat things aggressively if any problems come up."

References

Document

Pan American Health Organization (PAHO). (2008). *Caribbean Guidelines for the Scale Up of HIV Testing and Counselling Towards Universal Access*

Pan American Health Organization (PAHO). (2008). *Provider Initiated HIV Testing and Counseling One day Training Programme.*

UNAIDS/WHO. (2004). *UNAIDS/WHO Policy Statement on HIV Testing*

Book

JHPIEGO (2004). *Counseling and Testing for HIV.*

APPENDICES

Appendix 1

PITC PROTOCOL CHECKLIST

Instructions: Place a "√" in the spaces provided indicating that skill or task is satisfactorily or unsatisfactory performed. If task or skill is not observed write the letter "N"

CHECKLIST FOR USING THE PITC PROTOCOL TO PROVIDE HIV COUNSELING			
Task/ Skill	Satisfactory	Unsatisfactory	Not observed
1. Provide adequate pre-test information			
2. Follow the protocol, addressing the appropriate question/s			
3. Assure the individual of confidentiality			
4. Obtain informed consent in a professional manner			
5. Provide opportunity for the client to ask questions			
6. Use active listening skills			
7. Maintain eye contact			
8. Use language that the client understands			
9. Affirm the client when appropriate			

Appendix 2

PITC Role Plays

Role play 1

Your client is a 36yr.old married woman. She tells you she has a vaginal discharge. She also shares that she has had the discharge for over three days and her husband experiences painful urination. As a PITC counselor how do you proceed?

Role play 2

Mark is a MSM but has to keep it a secret. He has journeyed many miles to get to this clinic located at the hospital. It is not his first visit but it stills gets a little anxious when he visits for medical care. He finds it difficult sharing his health concerns with the nurse. He eventually shared with the nurse that ^{he} his has sores on his genital. As a PITC counselor how do you proceed?

Role play 3

Althea a teenager shared with you that she had sexual intercourse with a married man. Since then, she has developed swollen lymph glands and comes to the outpatient clinic of the hospital for consultation. As a PITC counselor how do you proceed?

